Grading Rubric for Calgary Cambridge-

Step 1: Build rapport & establish safety

Check if the clinician:

☐ Introduces themselves and their role

☐ Uses or confirms the patient's preferred name and/or pronouns

☐ Uses a welcoming or reassuring opening statement
- *"I'm glad you came in today."*

☐ Acknowledges patient as a person
- *"That sounds like it's been really frustrating for you."*

☐ Signals openness to listen
- *"Take your time — I want to hear what's been going on."*

Step 2: Set the agenda collaboratively

☐ Open-ended question about concerns
- *"What brings you in today?"*

☐ Invites additional concerns
- *"What else has been bothering you?"*

☐ Allows multiple concerns before narrowing
- *"Okay, chest pain, fatigue, and trouble sleeping — anything else?"*

☐ Negotiates priorities
- *"Which of these feels most urgent to start with?"*

☐ Confirms agenda
- *"So today we'll focus on the chest pain and talk about sleep if time allows."*

Step 3: Invite the patient's narrative

☐ Open-ended narrative prompt
- *"Can you walk me through what first started happening?"*

☐ Minimal interruption early on

☐ Continuers/encouragers
- *"Mm-hmm." / "Go on." / "I see."*

☐ Allows silence or pauses
- *(Brief pause) "Take your time."*

☐ Responds to narrative rather than redirecting
- *"You mentioned this started after the move — tell me more about that."*

Step 4: Structured exploration with explicit signposting

☐ Explicitly announces a transition to structured questions
- *"I'm going to ask you some more specific questions now."*

☐ Names the topic or domain before questioning
- *"I want to ask a few questions about your pain."*

☐ Uses sequential or ordered language (e.g., "first," "next," "now")
- *"First, let's talk about when this started."*
- *"Next, I'd like to ask about how this affects your daily life."*

☐ Frames questions as part of an organized assessment
- *"To get a clearer picture, I need to ask about your sleep and stress levels."*

☐ Maintains clinician control of topic flow
- *"We'll come back to that in a moment — right now I want to focus on your medications."*

Step 5: Summary & check-back

☐ Summarizes medical content
- *"So the headaches started about three months ago and happen mostly in the mornings."*

☐ Summarizes emotional/personal content
- *"And it sounds like they've been making work really hard and increasing your stress."*

☐ Reflective/paraphrased language
- *"What I'm hearing is that this has been building up over time."*

☐ Checks accuracy
- *"Did I get that right?"*

☐ Invites correction or additions
- *"Is there anything important I missed?"*

Step 6: Closing, shared plan & next steps

☐ Signals the visit is moving toward closure
- *"Before we wrap up today…"*
- *"Let's talk about next steps."*

☐ Proposes or confirms a plan of action
- *"I'd recommend starting physical therapy and adjusting your medication."*

☐ Invites patient input or agreement on the plan
- *"How does that plan sound to you?"*
- *"Does that feel manageable?"*

☐ Explains next steps or follow-up logistics
- *"We'll schedule a follow-up in four weeks, and I'll call you with the lab results."*

☐ Checks understanding or readiness before ending
- *"Can you tell me what the plan is so I know we're on the same page?"*
- *"Any questions before we finish?"*